جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شهيره فاروق ادوار حبيب

المحافظة : القاهره **الحي / المدينة :** الشرابيه **العنوان :** 28 ش العريان الشرابيه

رقم البطاقة : 27612140100088 **رقم التليفون :** 01208269017 **تاريخ تسجيل الإستمارة :** 20-10-2026

العمر(تاريخ الميلاد) : 14-12-1976 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شهيره فاروق ادوار حبيب
National ID: 27612140100088

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : شهيره فاروق ادوار حبيب
Age : 49 Y : 3 M : 2 D **Gender :** Female
Referred By Prof. Dr. : رانيا الاحمدى
Clinical Diagnosis : Right breast UOQ mass
Nature of Specimen : US guided tru cut biopsy
Receiving Date : 2026-04-07

Analysis ID : 1046641
Conf. Code : 9109
Patient ID : 767504

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue cores, longest measured 1 cm, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue formed of malignant ductal epithelial cells, showing moderate to marked nuclear pleomorphism and hyperchromasia.
There is poor attempt at tubular formation with solid sheets.
There are moderate number of mitotic figures.
There is marked desmoplasia.
There is insignificant peritumoural lymphocytic infiltrate.
There is focal necrosis.
No intraductal component.

Diagnosis :

Right breast mass, US guided tru cut biopsy, INFILTRATING DUCT CARCINOMA, GRADE III.
- HORMONAL RECEPTORS IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

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DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠٥٢١٧٠٠١٢٨٧

تم المسح الضوئي بـ CamScanner
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: شهيره فاروق ادوار حبيب	
MR. : 10674	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presents with right-sided breast cancer, clinically staged as cT3-cT4 N1 M0 (MDT cT2). Imaging reveals a bulky left adrenal gland metastasis; the disease is otherwise free of distant sites. Pathology shows an IDC grade 3, triple-negative tumor with Ki-67 60 % and HER2-negative (not amplified). Physical examination demonstrates a right breast mass at the 12 o'clock position measuring approximately 10 × 10 cm, with a 2 × 2 cm right axillary lymph node. The overlying skin exhibits peau d'orange changes covering about 65 % of the mass. The patient's weight is 90 kg, height 158 cm, and calculated BSA is 1.98 m². The initial treatment plan includes weekly Taxol 155 mg/m² and Carboplatin AUC 2. The patient has completed week 1 of this Taxol + Carboplatin regimen.

The treatment was continued with cycle 2 week 1 of Taxol 155 mg/m² and Carboplatin AUC 2, together with cycle 1 of Keytruda. CBC results were accepted, while chemistry labs were pending; pharmacy preparation for weeks 2 and 3 was arranged. The patient has completed cycle 2 week 1 of the Taxol + Carboplatin regimen and cycle 1 of Keytruda.

During the pharmacy visit for cycle 2 week 2, the patient's weight, height, and BSA remained unchanged (90 kg, 158 cm, BSA 1.98 m²). Labs dated 24/8 were reviewed and accepted. The patient has completed cycle 2 week 2 of the Taxol + Carboplatin regimen.

At the most recent pharmacy encounter for cycle 2 week 3, the patient's weight, height, and BSA remain stable (90 kg, 158 cm, BSA 1.98 m²). Laboratory results from 24/8 were accepted. The patient has completed cycle 2 week 3 of the Taxol + Carboplatin regimen.

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	

الطبيب المعالج : dina makram

يعتمد ,

Dr.
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Dr.
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Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : شهيره فاروق انوار حبيب
Age : 49 Y : 3 M : 2 D **Gender :** Female
Analysis ID : 1046642
Conf. Code : 9871
Referred By Prof. Dr. : رانيا الاحمدى
Clinical Diagnosis : Right breast UOQ mass
Nature of Specimen : Block [labelled as 1046641]
Receiving Date : 2026-04-07
Patient ID : 767504

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block then treated by monoclonal antibodies and detection kit (EnVision FLEX) using DAB as chromogen and hematoxylin as counter-stain, using Omnis machine.
The sections were treated against:
- Estrogen receptors - Progesterone receptors
Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.
The sections were treated against:
- Her 2 - Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.
Sections treated for Progesterone receptors revealed NEGATIVE nuclear staining.
Sections treated for HER2 revealed MODERATE CONTINUOUS CELL MEMBRANE STAINING IN MORE THAN 10% OF TUMOR CELLS.
60% of Tumor cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE
NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8)
NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8)
HER 2 EQUIVOCAL, SCORE 2+
CASE FOR CONFIRMATION OF HER 2 GENE AMPLIFICATION BY SISH TECHNIQUE.
THE PROLIFERATION INDEX OF Ki67 IS 60%.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

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عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧

تم المسح الضوئي بـ CamScanner

Dr
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Member , United States Canadian Academy of Pathology

Patient Name : شهيره فاروق ادوار حبيب Analysis ID : 1051193
Age : 49 Y 4 M 8 D Gender : Female Conf. Code : 5772
Referred By Prof. Dr. : رانيا الاحمدى
Clinical Diagnosis : Right breast UOQ cancer.
Nature of Specimen : Block (labelled as 1046641 - US guided tru cut biopsy)
Receiving Date : 2026-04-23 Patient ID : 767504

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections prepared from the paraffin block were stained for HER 2 gene and chromosome 17 using Silver In Situ Hybridization (SISH) technique in Benchmark XT (Ventana).

Results :

The ratio of Her 2 signals to chromosome 17 signals in the nuclei of the tumour cells is 1.5 (less than 2) indicating absence of amplification of HER2 gene.
Average Her 2 copy number/tumor cell is 3,

Conclusion :

Block, HER 2 by SISH, NEGATIVE FOR HER 2 GENE AMPLIFICATION, RATIO IS 1.5.

Prof. Dr. Shady Elia Anis *Prof. Dr. Elia Anis Ishak*

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DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧

CamScanner
immunohistochemistry
[[DOCTYPE:IHC]]

alfa
L A B S

Collection Date: 06/04/2026
Review Date: 06/04/2026

Patient Name: الاستاذة شهيره فاروق الدوار
Patient No.: 128-40087
Accession No.: 12826042963
Age: 49Y

HEMATOLOGY UNIT

Complete Blood Picture (CBC)

Test Name	Result	Unit	Reference Interval
Haemoglobin	11.90	g/dL	12 - 15
Hematocrit	35.8	%	36 - 46
Red cell count	4.09	$x10^6$ /uL	3.8 - 4.8
MCV	87.5	fL	83 - 101
MCH	29.1	pg	27 - 32
MCHC	33.2	g/dL	31.5 - 34.5
RDW	15.8	%	11.6 - 14
Platelet Count	319	$x10^3$/uL	150 - 410
T.L.C	5.6	$x10^3$/uL	4 - 10

Differential Count	Percent Values		Absolute Values		
Basophils	1	% 0 - 2	0.1	$x10^3$/uL	0.02 - 0.1
Eosinophils	2	% 1 - 6	0.1	$x10^3$/uL	0.02 - 0.5
Stab	0	% 0 - 7	0.0	$x10^3$/uL	
Segmented	53	% 40 - 80	3.0	$x10^3$/uL	2 - 7
Lymphocytes	37	% 20 - 40	2.1	$x10^3$/uL	1 - 3
Monocytes	7	% 2 - 10	0.4	$x10^3$/uL	0.2 - 1
MPV	10.3	fL 8 - 12			

Comments : Mild normocytic normochromic anemia with mild anisocytosis.
Platelets are adequate.
WBCs are within normal limits.

Reviewed By:
Dr Nesrine Helal

Pt. Branch: العبور

Printed by: 4683حسام حسن السيد

معامل الفا
16191
AMG A MEMBER OF ALFA MEDICAL GROUP
تم المسح الضوئي بـ CamScanner

RIGHT BREAST:

- Upper midline, posterior third ill-defined speculated mass lesion, with peripheral micro calcifications.
- UOQ, posterior third two ill-defined focal asymmetry, with no underlying lesions.
- This was confirmed by **3D Tomosynthesis 1mm section images.**
- **Complementary ultrasound** revealed 12-1 o'clock, zone c irregular angulated hypoechoic mass lesion (correspond to mammographic finding), measures 23 x 22 mm, seen at 20 mm from overlying skin.
- **UOQ focal adenosis (correspond to mammographic finding) with subtle distortion, for further MRI evaluation**
- Normal skin/nipple/areolar complex.
- Preserved axillary lymph nodes.

Best Regards,
Dr. RANIA AHMED, MD

تم المسح الضوئي بـ CamScanner

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name : Mrs. Shahira Farouk Adwar

Date : 16/03/2026 Code : 3517295
Room Name : U.S - Semines - HE

3D FULL FIELD DIGITAL MAMMOGRAPHIC EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY: left breast lump.
IMPRESSION:
LEFT BREAST: (BIRADS 2)
- 1 o'clock IMLN

RIGHT BREAST: (BIRADS 4B)
- 12 o'clock suspicious mass lesion
- UOQ focal asymmetry? focal adenosis, for further MRI evaluation

RECOMMENDATION:-
- For sonar guided biopsy
- MRI breast

DETAILED FINDINGS:

TECHNIQUE:
- Craniocaudal & Mediolateral Oblique views
- Tomosynthesis in 1mm slice thickness
- High resolution ultrasound with color doppler

Both breasts showed heterogeneously dense breasts, (ACR c).

LEFT BREAST:
- No dominant mass lesion, spiculations or suspicious microcalcifications.
- UOQ oval circumscribed density
- This was confirmed by **3D Tomosynthesis 1mm section Images**
- **Complementary ultrasound** revealed 2 o'clock, zone c , oval IMLN, with preserved hilum , measures 10 x 6 mm .(correspond to mammographic finding)
- No signs of malignancy detected.
- Normal skin/nipple/areolar complex.
- Preserved axillary lymph nodes.

تم المسح الضوئي بـ CamScanner

Mammogram
[[DOCTYPE:MAMMOGRAM]]

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LABS

Collection Date: 06/04/2026	**Patient Name:** الاستاذة/ شهيره فاروق الدوار
Review Date: 06/04/2026	**Patient No.:** 128-40087
	Accession No.: 12826042963
	Age: 49Y

HEMATOLOGY UNIT			
Test Name	**Result**	**Unit**	**Reference Interval**
Prothrombin Time and Conc.			
Control	**12.3**	Seconds	
Patient Time	**11.6**	Seconds	
Concentration	**113**	%	70 - 140
INR	**0.94**		Less Than 1
Partial Thromboplastin Time (PTT).			
Partial Thromboplastin Time	**27.1**	Seconds	24 - 36
Bleeding Time	**2**	min	1 - 4
Clotting Time (Lee-White)	**8**	min	4 - 10

Reviewed By:
Dr.Nesrine Helal

Pt. Branch: الحرفاني

Printed by: 4683حسام حسن السيد

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AMG ALFA MEDICAL GROUP A MEMBER OF ALFA MEDICAL GROUP
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تم المسح الضوئي بـ CamScanner
Lab Test
[[DOCTYPE LAB_TEST]]

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SCAN

Name: Shahira Farouk Adoard *ID: 5103333*
Referred by: Prof. Rania Mohamed Al Ahmady *Date: 01/04/2026*

High Field Open (1.5T) Dynamic MR-Mammography of Both Breasts

Technique :

- Axial T2 (plate 1).
- Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5).
- Time intensity curves (plate 6).

Findings:

- Bilateral glandular condensation and evidences of bilateral homogenous glandular enhancement.
- Right breast UOQ 12:00 location deeply seated lobulated outline mass with fine speculation , it measures 30x25 mm in diameters , the lesion is low signal in T2 and STAIR with intense mass like enhancement with ductal extension. The kinetic data obtained indicates a suspicious pattern , rapid rise followed by wash out and plateau , type 2& 3 curves .
- No left breast masses and no architecture distortion.
- Normal skin thickness and contour of both breasts .
- There is multiple likely non-specific bilateral axillary lymphadenopathy.

OPINION :

Right breast UOQ suspicious mass ,categorized as BIRADS 4C based on the morphological and the kientic data.Microbiopsy is recommended.
Normal study of the left breast , BIRADS 1.

Prof. Naglaa Abdel Razek.MD

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Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	0 %
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	2 %
BIRADS 3 : Probably benign.	50 %
BIRADS 4 : Probably malignant.	
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	

الفاسكانر

تم المسح الضوئي بـ CamScanner

Mammogram
[[DOCTYPE MAMMOGRAM]]

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SCAN

ID: 5103333 Name: Shahira Farouk Adoard
Date: 07/04/2026

Breast Biopsy Report

US guided true cut biopsy from a right breast UOQ mass radiologically classified as R4 (suspicious)*.
Multiple cores were taken using a 14g needle and the tissue is sent for :

- Histopathology and hormonal receptors.

Kindly accept my best regards,
Dr. ENGY ADEL, M.D.

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions :

BIRADS 0 : needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2 : Benign .
BIRADS 3 : Probably benign.
BIRADS 4 : probably malignant.
BIRADS 5 : Malignant.
BIRADS 6 : Proven pathologically malignant.





تم المسح الضوئي بـ CamScanner

جوستاف روسي
انترناشونال

إسم المريض ثلاثي: .....

رقم الملف: .....

تقريـر طبـــى

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشان تقرير طبي للمريض: شيرين فاروق

بقسم.....رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... .....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

Breast Ca

Triple negative

Received 10 weeks Taxol/Carboplatin

week 10 on 19/10/2026

استكمال 2 اسابيع

تحريرا في

الوقت: ..... التاريخ : / /

الطبيب المعالج

.....

يعتمد ،،